

From Insights to Impact

*Unpacking Disparities and Driving
Community-Centered and Research Solutions*

Meredith McGee *(She/Her)*

Yale LGBTQ Mental Health Initiative

Guilford Family and Youth Services

December 4th, 2024



Presentation Roadmap

01

**Who We Are and
What We Do**

02

**LGBTQ+ Population
Health:
Foundations and
Insights**

03

**Unpacking Mental
Health Disparities
in LGBTQ+
Communities**

04

**Addressing
Disparities
Through Research**

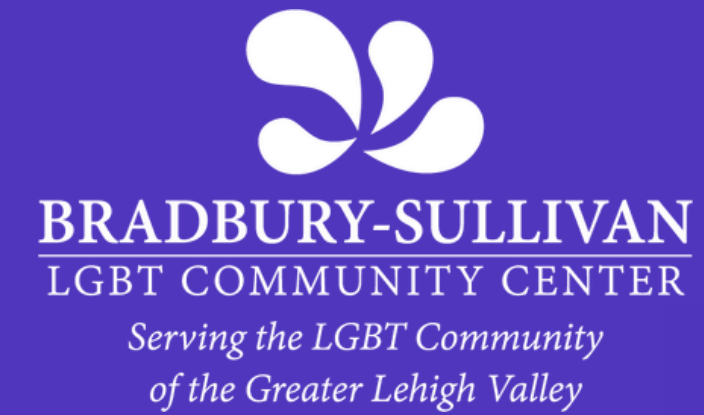
05

**Expanding
Reach into the
Community**

01

Who We Are and What We Do

About Me



Graduate Research Assistant, Public Health Student
*LGBTQ+ health, Substance Use + Harm Reduction,
Community-Based Health*



Yale LGBTQ Mental Health Initiative

The Yale LGBTQ Mental Health Initiative is devoted to **understanding and improving the mental health of LGBTQ+ populations by:**

1. Assembling a **collaborative, multidisciplinary investigative team** within and beyond Yale to **conduct rigorous and relevance research**
2. Examining the **mechanisms underlying LGBTQ+ health disparities** to advance theory, identify
3. targets for intervention, and advocate for improved care
4. Developing and testing **LGBTQ-affirmative interventions to improve LGBTQ+ mental and behavioral health**
5. Disseminating our LGBTQ-affirmative treatments through **training of mental health providers and implementation research**
6. Culturally adapting and expanding our interventions to meet the **mental health needs of local and global LGBTQ+ communities**



Our Team



John Pachankis, PhD
Professor of Public Health
Clinical Psychologist



Skyler Jackson, PhD
Asst. Professor of Public
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Counseling Psychologist



Danielle Chiaramonte, PhD
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Clinical Psychologist



Tyler Harvey, MPH
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Epidemiologist



Research Associates
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Our Collaborators



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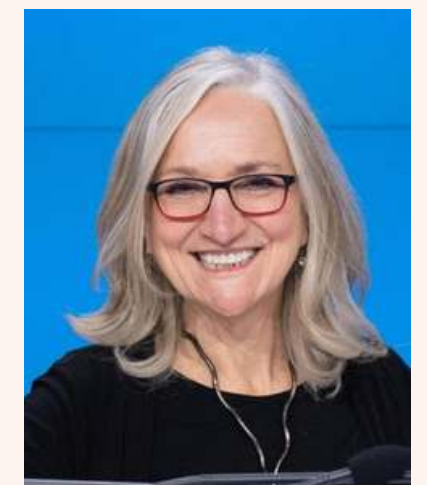
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CenterLink
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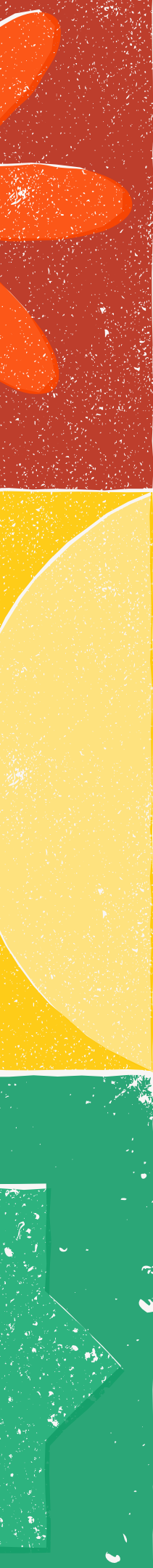
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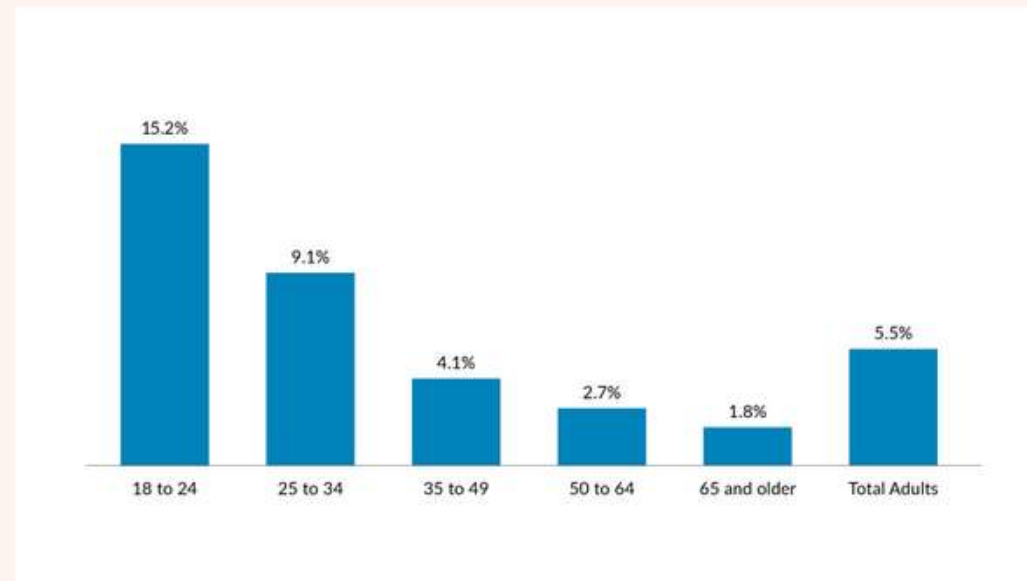
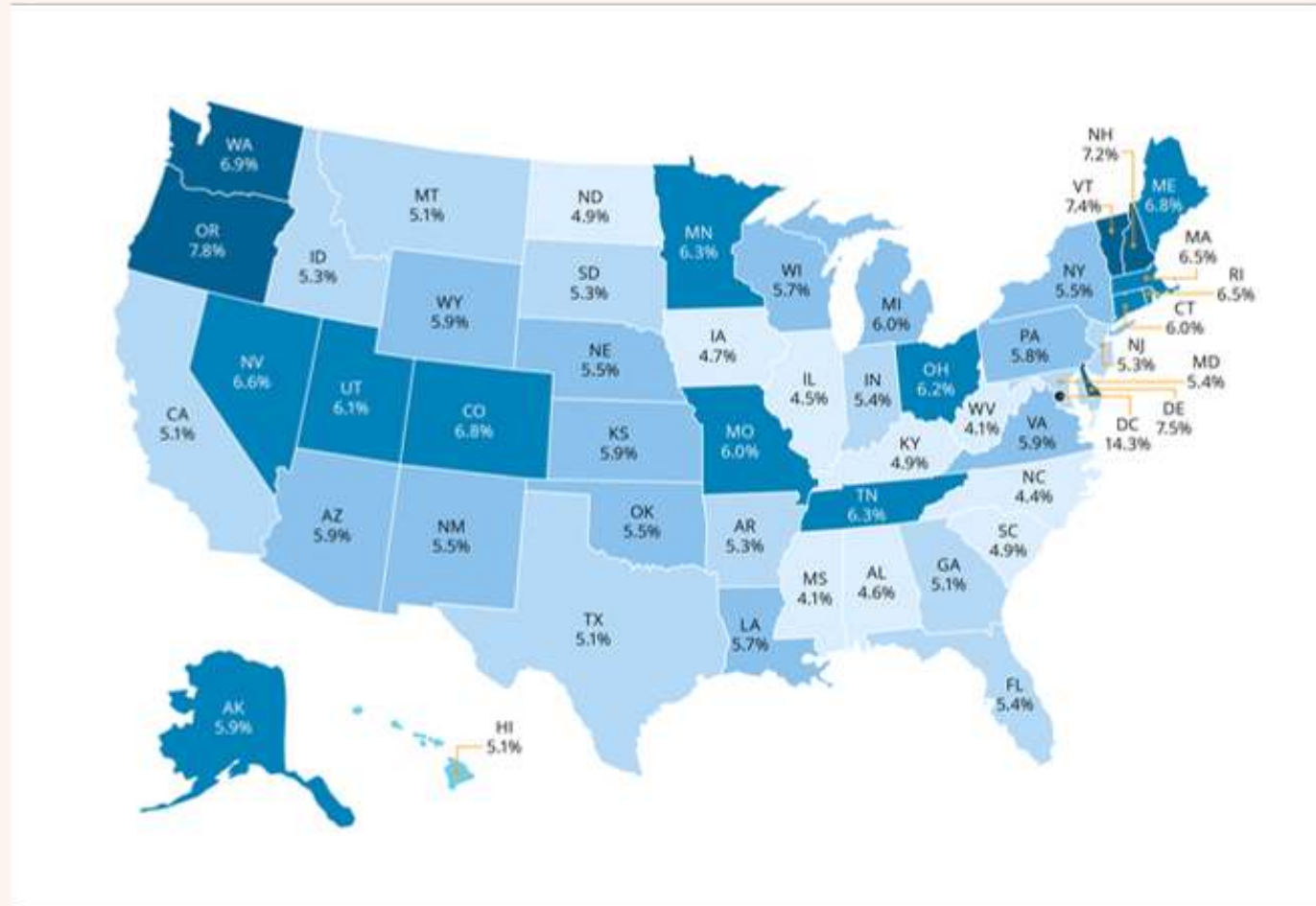
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**Expanding
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02

LGBTQ+ Population Health: Foundations and Insights



United States:

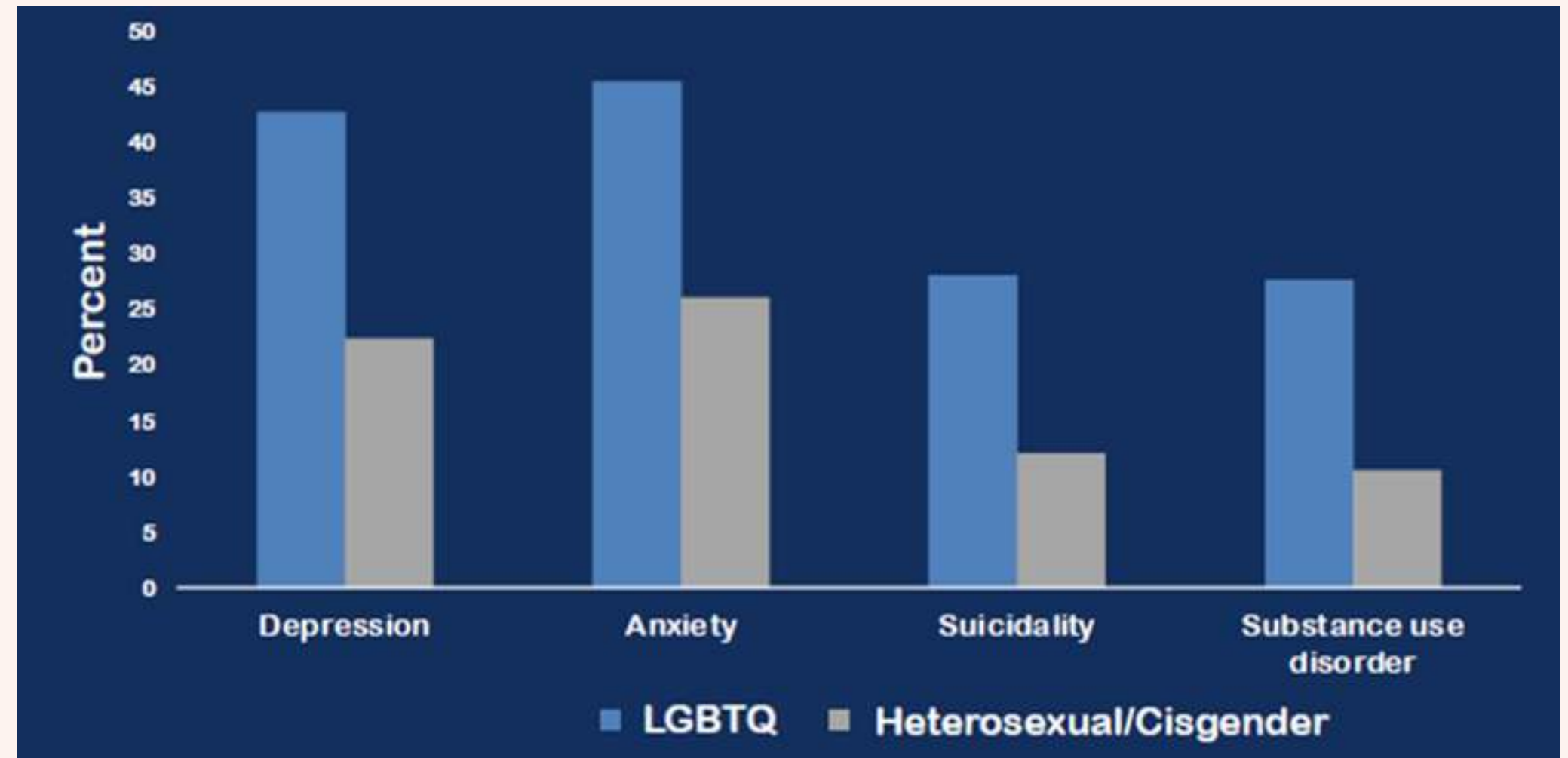
- **5.5%** of U.S. adults identify as LGBTQ+ (approximately 13.9 million people)
- The **18–24 age group** has the highest representation, with 15.2% identifying as LGBTQ+
- An estimated **1.6 million people** ages 13 and older identify as Transgender
- An estimated **7–9%** of youth across the U.S. identify as LGBTQ+*

Connecticut:

- **6.0%** of Connecticut adults identify as LGBTQ+ (approximately 170,500 people)
- An estimated **22,000** LGBTQ+ youth live in Connecticut.

State of LGBTQ Mental Health

- LGBTQ+ individuals are **twice as likely** to experience:
 - Depression
 - Anxiety
 - Suicidality
 - Substance Use Disorders
- Among the **highest-risk groups** in psychiatric epidemiology.
- Disparities emerge **early in life** and **persist globally**.



Swedish National Public Health Survey; US National Epidemiologic Survey of Alcohol and Related Conditions; California Quality of Life Survey; TransPop; Generations

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Unpacking Mental Health Disparities in LGBTQ+ Communities

What Drives These Disparities?

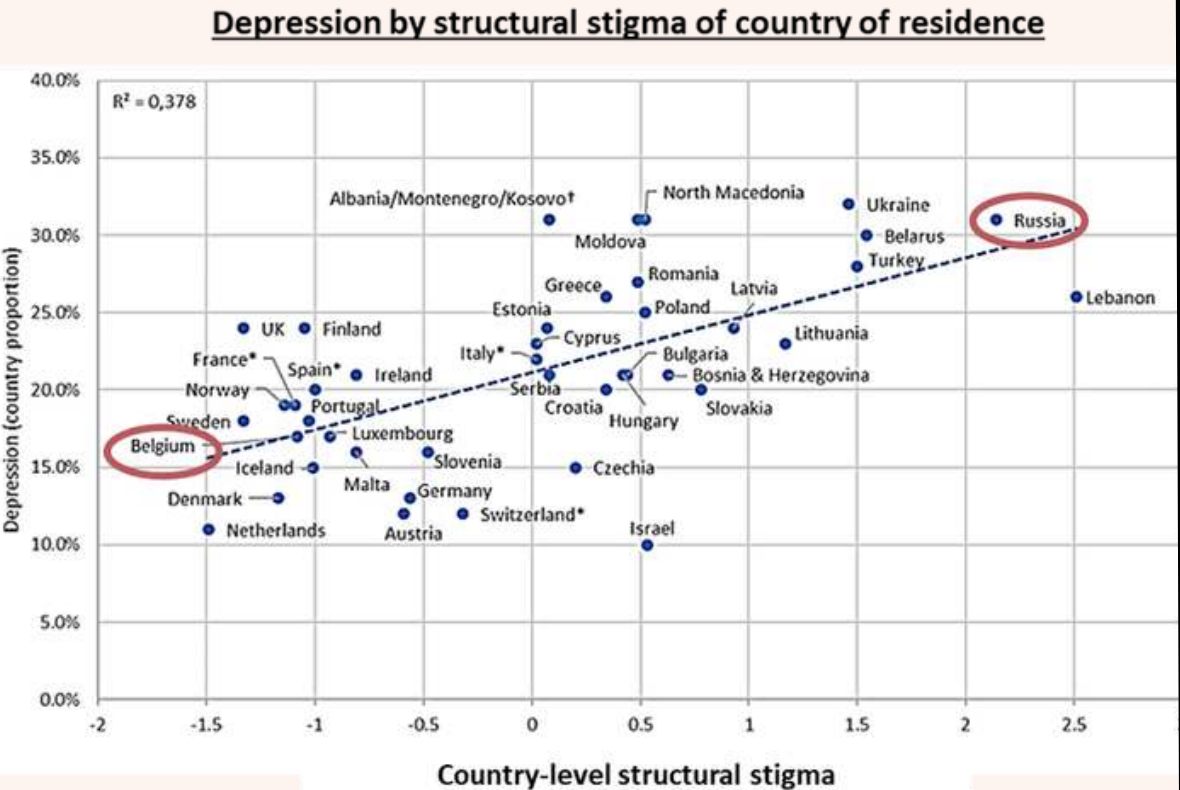
Stigma

Structural Stigma

Societal norms, policies, and institutional practices that marginalize LGBTQ+ individuals and restrict access to resources and opportunities.

Manifestations

- Discriminatory Laws and Policies
- Healthcare Barriers
- Institutional Norms



Interpersonal Stigma

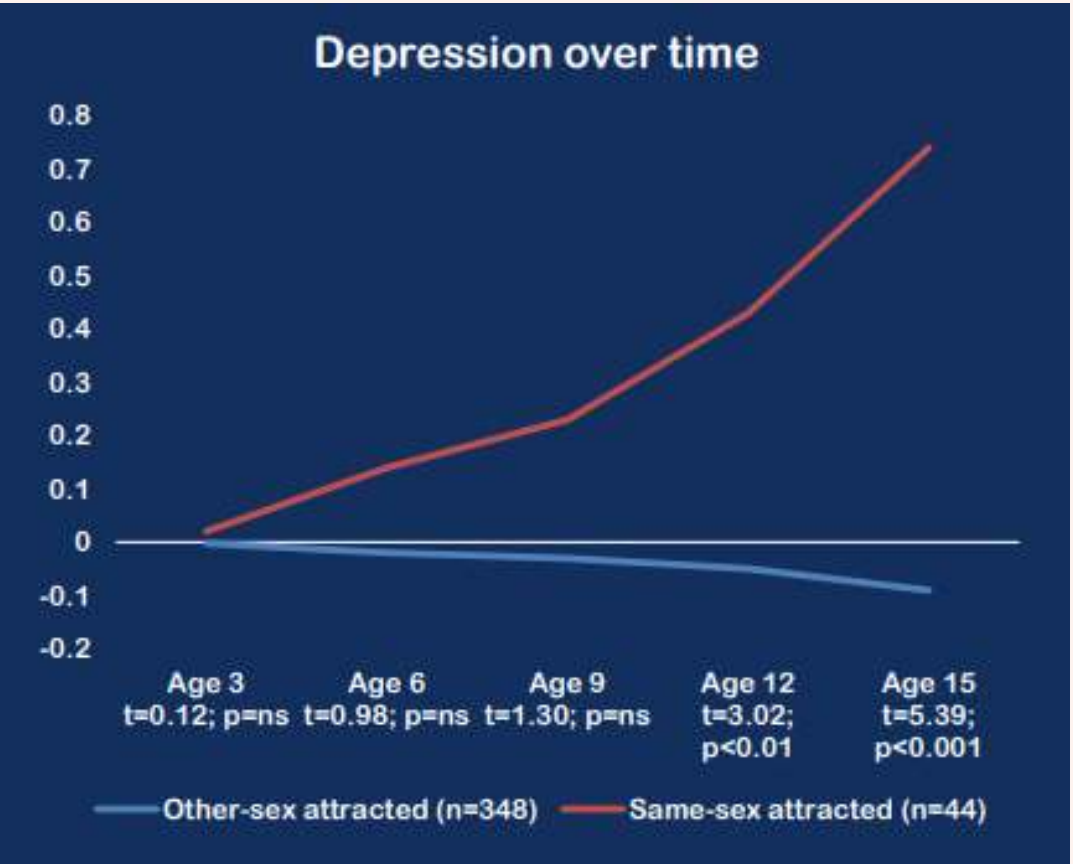
- **Knowledge** (Ignorance or Misinformation)
 - Limited or inaccurate understanding of LGBTQ+ identities
- **Attitudes** (Negative Emotional Reactions)
 - Prejudiced feelings such as discomfort, fear, or hostility toward LGBTQ+ individuals.
 - Often rooted in societal norms.
- **Behaviors** (Avoidance or Rejection)
 - Observable actions that discriminate, exclude, or harm LGBTQ+ individuals.
 - Examples include verbal insults, physical distancing, or outright rejection in social or professional settings.

Discrimination:

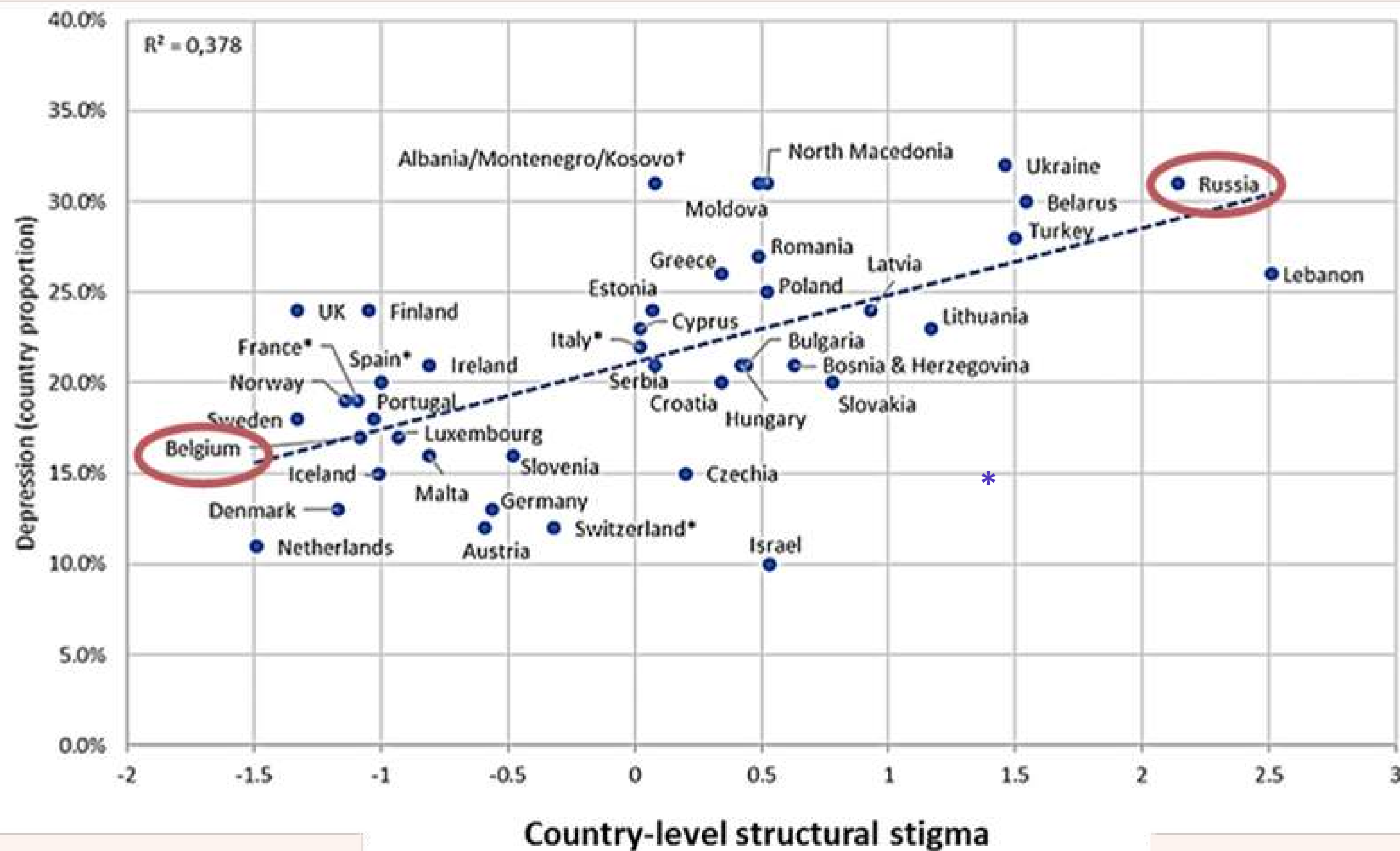
Bullying and harassment strongly predict depression and suicidal ideation

Parental rejection

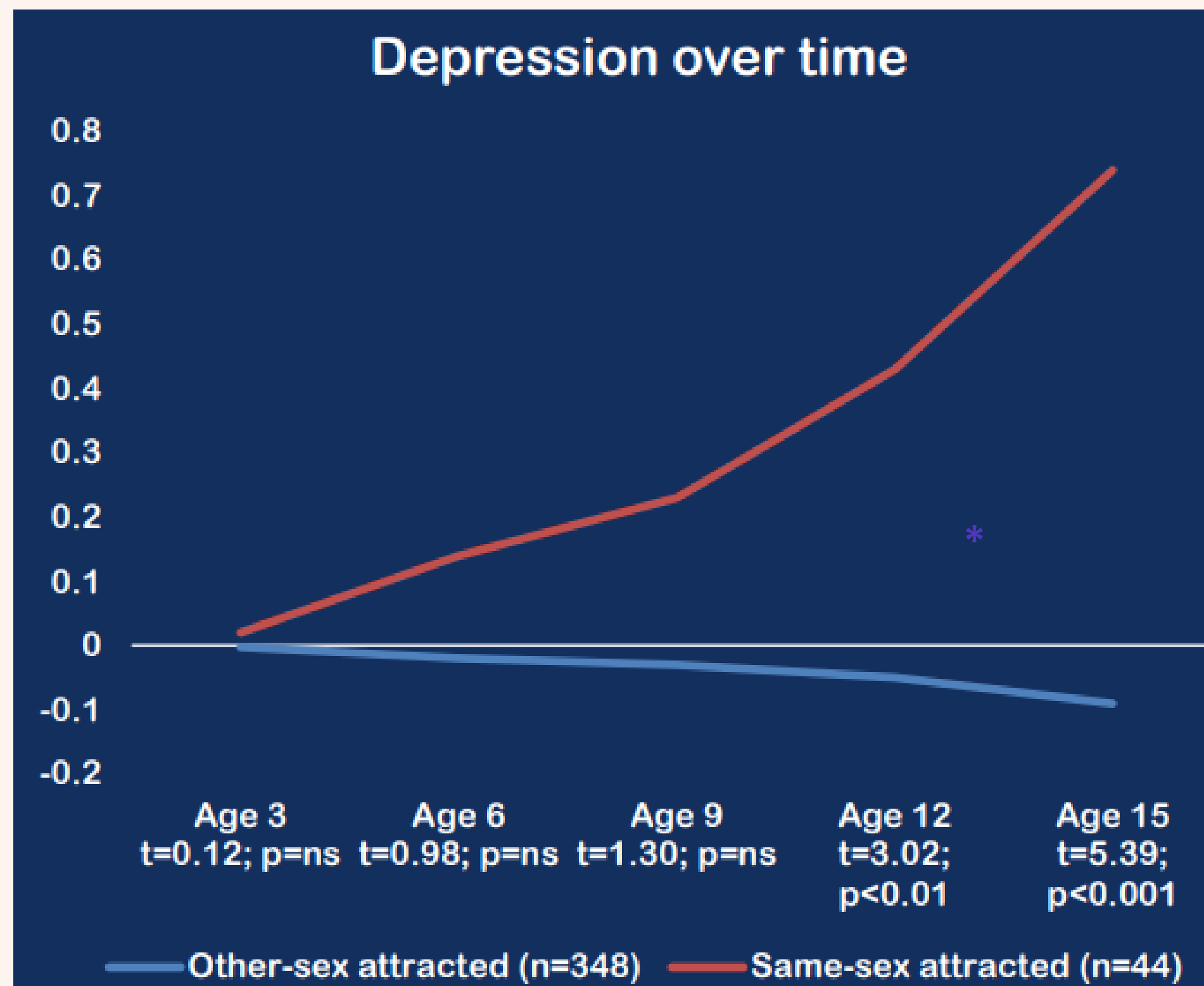
undermines a key protective factor for mental well-being, leading to increased isolation and vulnerability to external stigma



Depression by structural stigma of country of residence



Interpersonal Stigma

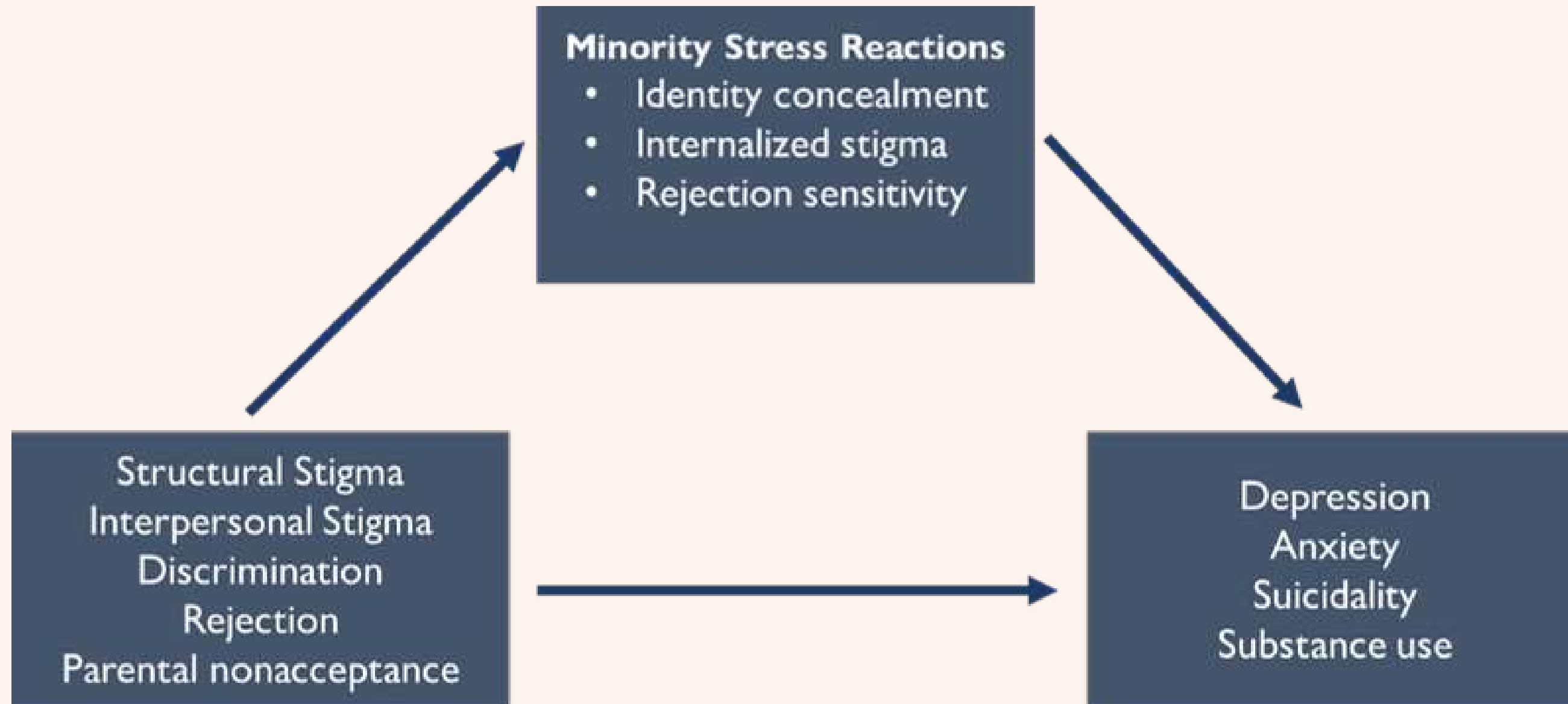


Minority Stress

Explains how stigma, prejudice, and discrimination create chronic stress unique to marginalized groups, including LGBTQ+ individuals.

- **Key Components:**

- Distal Stressors: External events like discrimination, rejection, or violence.
- Proximal Stressors: Internalized stigma, identity concealment, or fear of rejection.



Identity Concealment

The deliberate act of hiding or not disclosing one's LGBTQ+ identity to avoid stigma, discrimination, or negative social repercussions.

Impacts of Concealment

- Temporary Shield from Discrimination
- Chronic Hyper-Vigilance
- Heightened Self-Shame
- Social Isolation
- Increased Psychological Distress



Internalized Stigma and Shame

Negative beliefs about one's LGBTQ+ identity, leading to self-invalidation and feelings of inferiority.



Aspect	Shame	Stigma
Source	Internal, self-evaluative	External, societal
Focus	"I am bad" (self-invalidation)	"They see me as bad" (social rejection)
Mental Health Role	Directly linked to depression and anxiety	Creates conditions for emotional harm
Measurement	Self-reports (e.g., shame scales)	Observational or experiential (e.g., bullying, discrimination)

Rejection Sensitivity



The tendency to anxiously anticipate, perceive, and overreact to rejection, shaped by past experiences of stigma and discrimination as an LGBTQ+ individual.

Impacts of Rejection Sensitivity

- Difficulty forming and maintaining relationships
- Increased Psychological Distress
- Behavioral Avoidance
- Over-Interpretation of Social Cues

**Knowing this all, how do we improve
LGBTQ+ mental health?**

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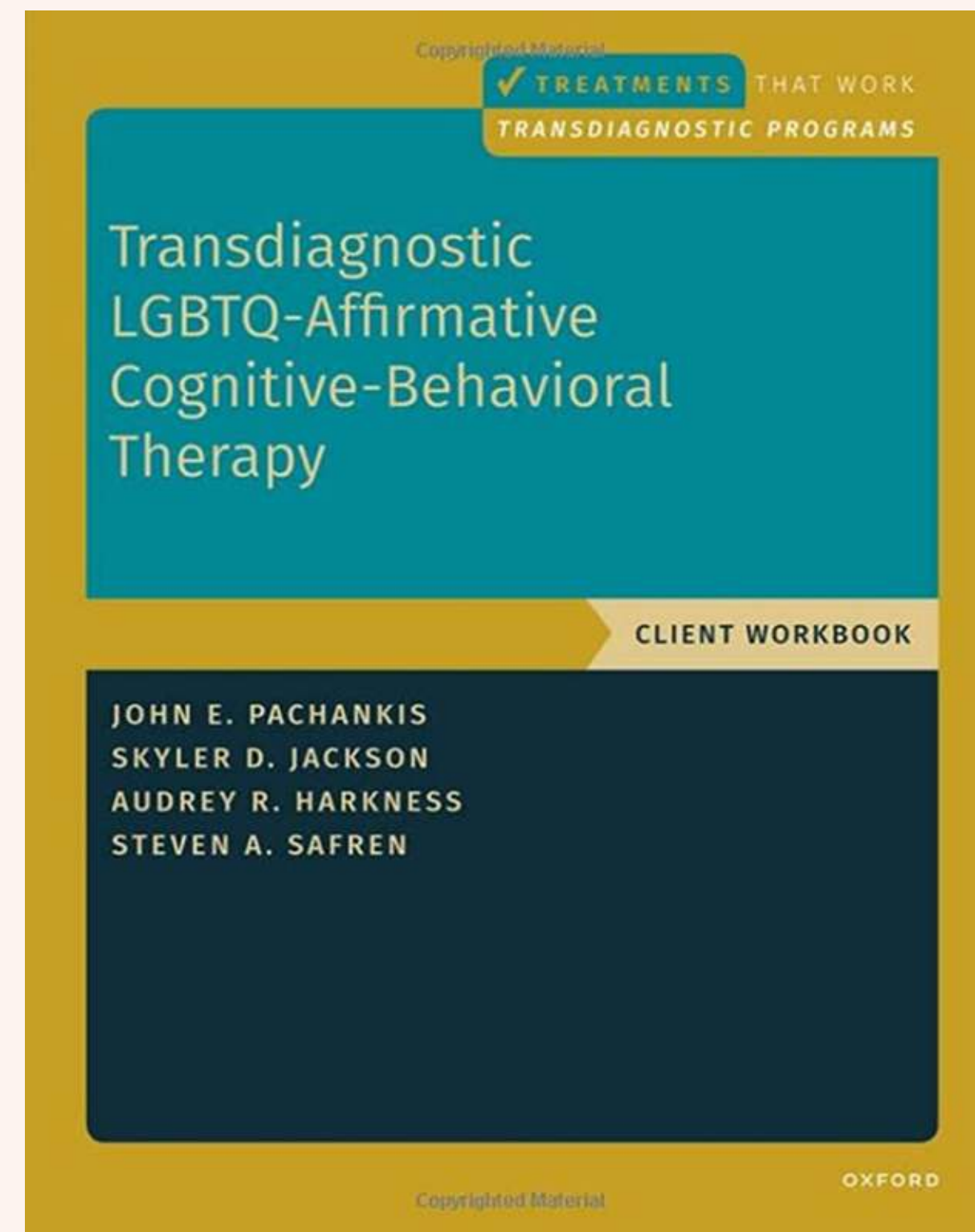
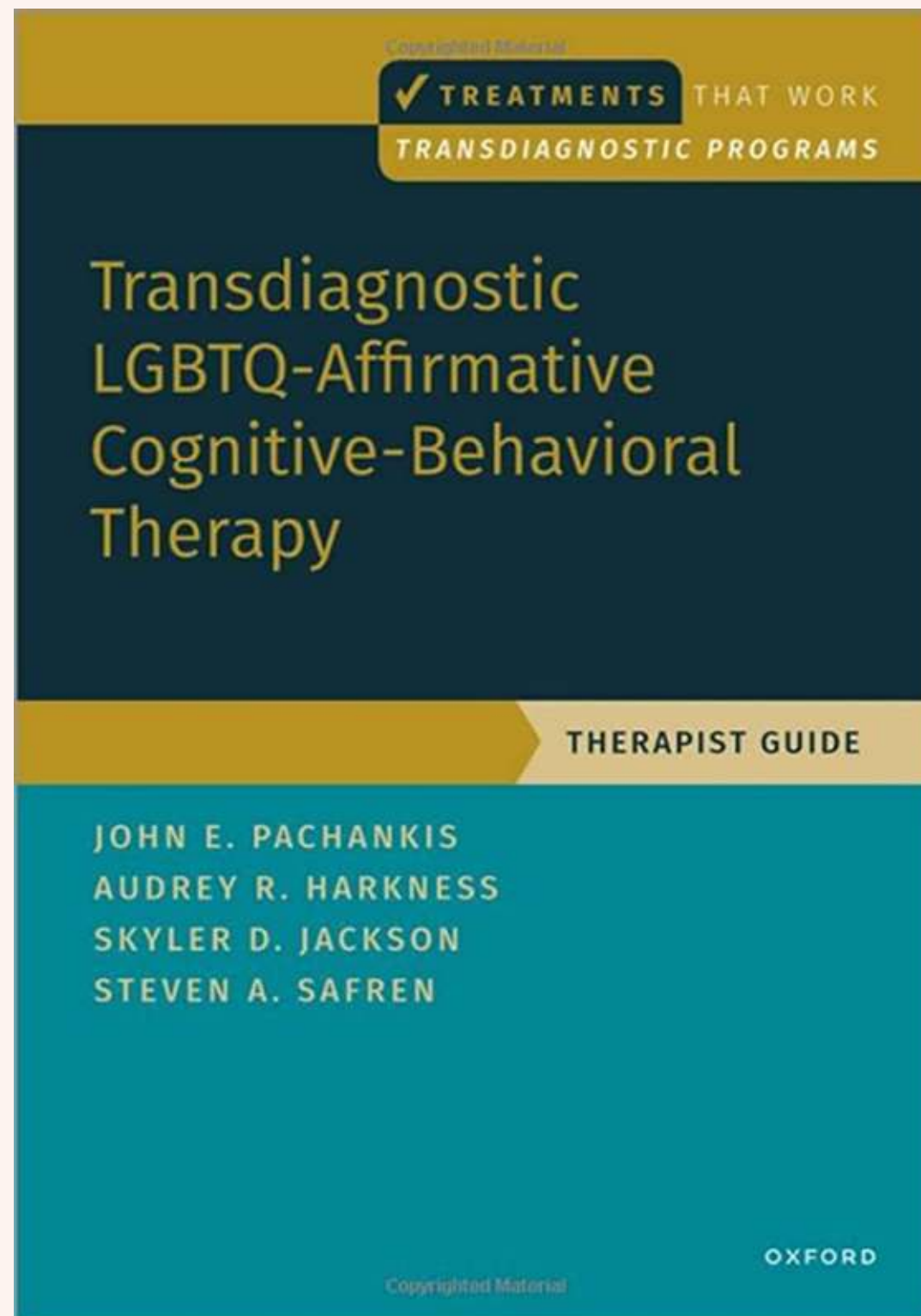
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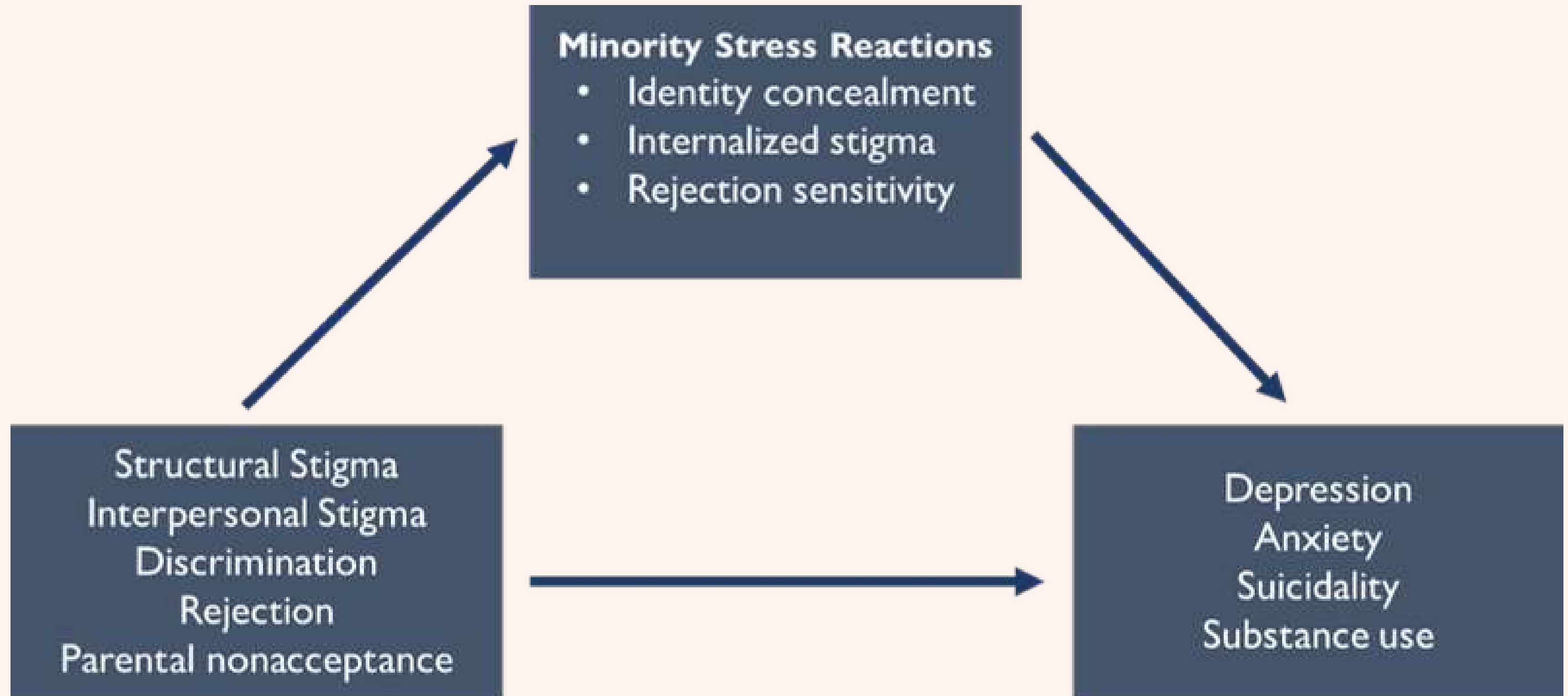
**Expanding
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Addressing Disparities Through Research

LGBTQ+ Affirming Cognitive-Behavioral Therapy





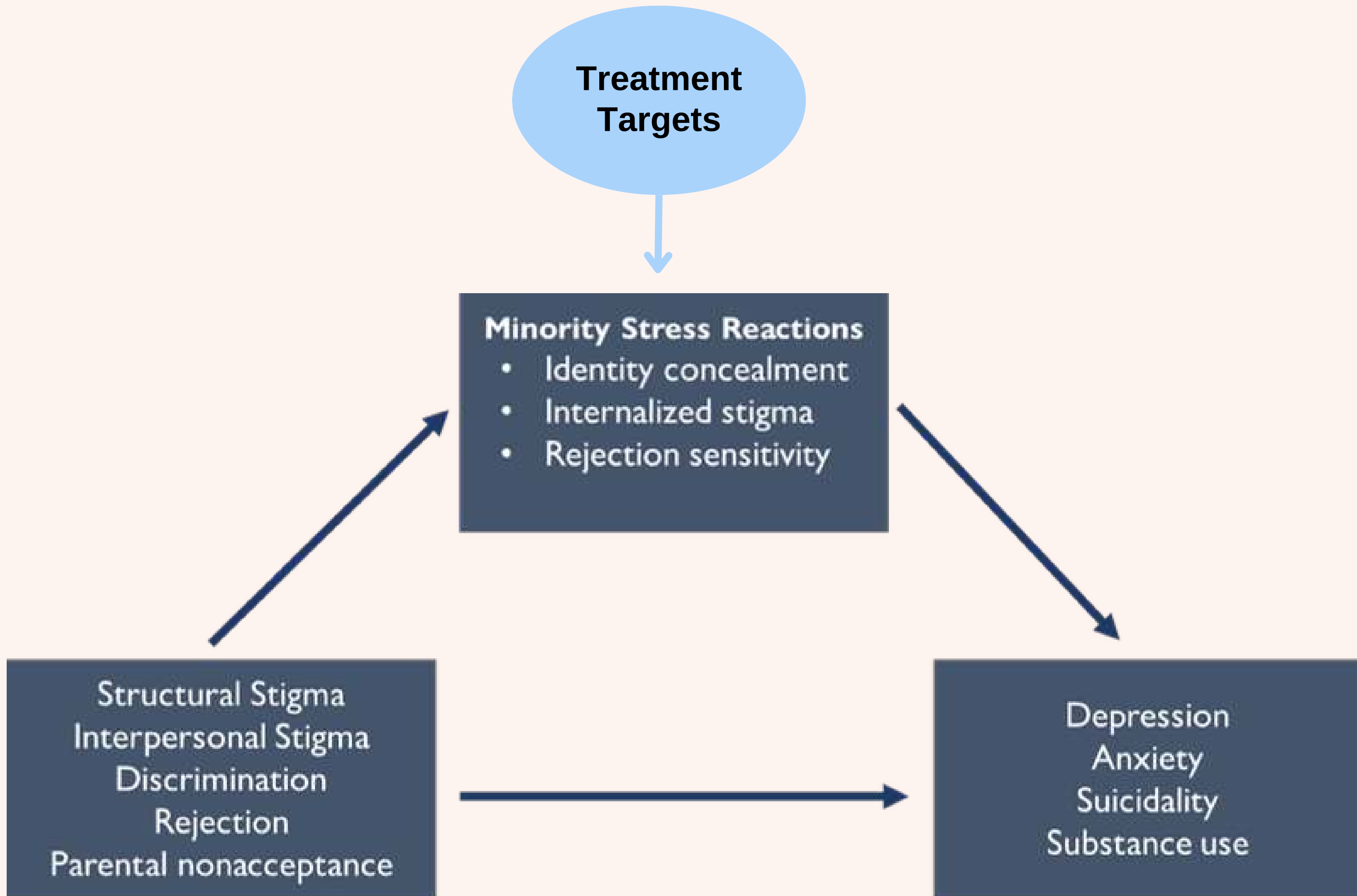
Treatment Targets

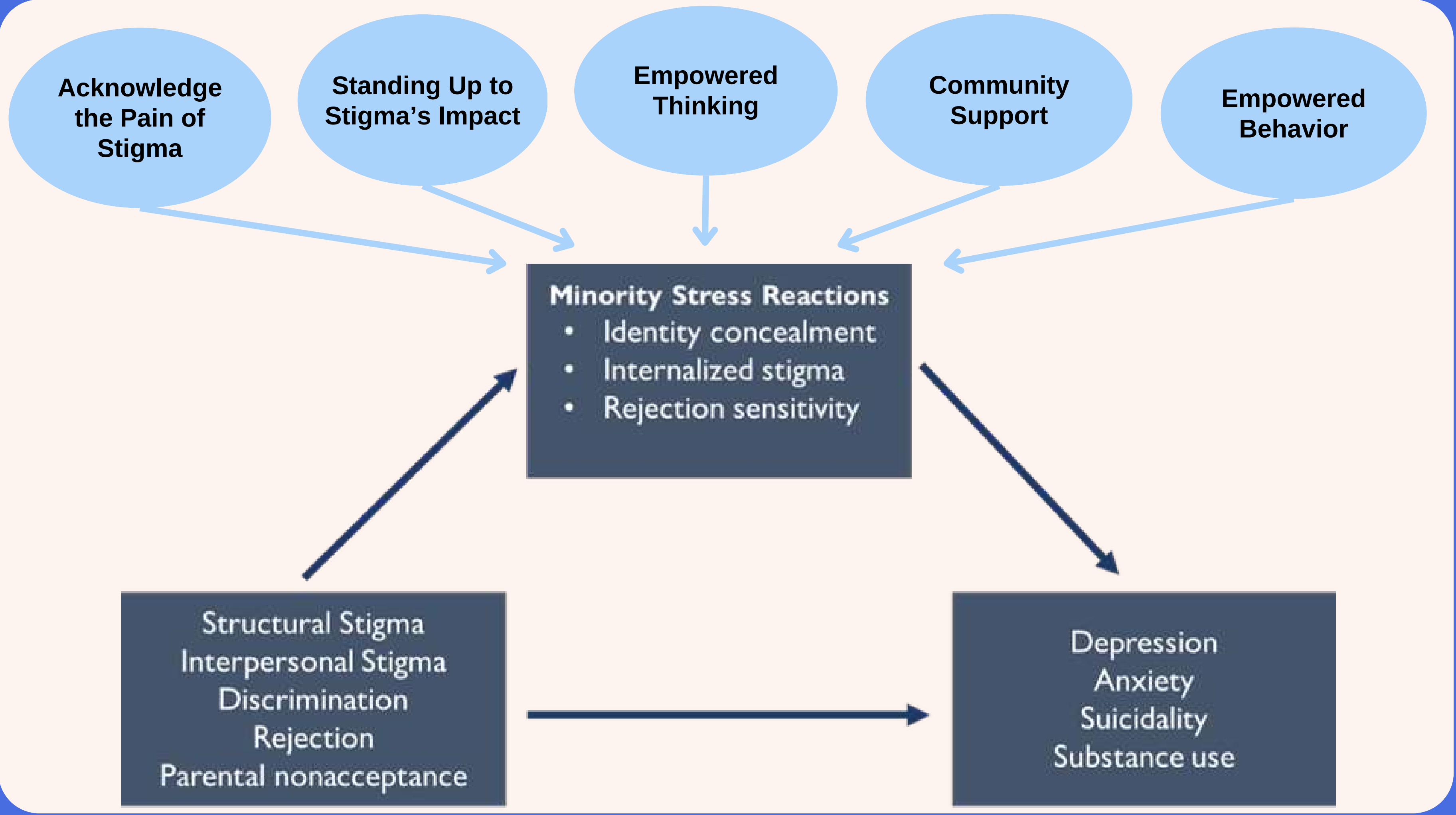
Minority Stress Reactions

- Identity concealment
- Internalized stigma
- Rejection sensitivity

Structural Stigma
Interpersonal Stigma
Discrimination
Rejection
Parental nonacceptance

Depression
Anxiety
Suicidality
Substance use





Treatment Format

9 modules

- Each contains a number of recommended sessions
- Order based on skills building on one another and progression of change strategies
- Session sequence flexible

50-minute sessions

Each generally following this order:

1. Review progress monitoring,
- 2.Review of the home practice
- 3.Module content
- 4.Agree upon home practice

LGBTQ-Affirmative CBT (9 Modules)	
Session 1	Functional Analysis (STAIRCASE) and Introduction to Treatment
Module 1	Setting Goals and Building Motivation for LGBTQ-affirmative CBT
Module 2	Understanding the Nature and Emotional Impact of LGBTQ-Related Stress
Module 3	Understanding and Tracking LGBTQ-Related Stress and Emotional Experiences
Module 4	Increasing Mindful Awareness of LGBTQ-Related Stress Reactions
Module 5	Increasing Cognitive Flexibility
Module 6	Countering Emotional Behaviors
Module 7	Experimenting with New Reactions to LGBTQ-Related Stress
Module 8	Emotion Exposures for Countering LGBTQ-Related Stress
Module 9	Recognizing Accomplishments and Looking to the Future

LGBTQ-Affirmative CBT Trials

Gay and Bisexual Men: Full Trial

Effective **S**kills to **E**mpower **E**ffective
Men

(**ESTEEM**)



Design: Randomized Controlled Trial

LGBTQ-
Affirmative
CBT

vs

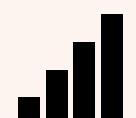
HIV Testing
and
Counseling

vs

Community
Therapy



Population: 254 gay and bisexual men in
NYC



Findings: Reductions in

- Depression
- Anxiety
- HIV Transmission

Queer Women: Full Trial

Empowering Queer Identities in
Psychotherapy
(**EQulP**)



Design: Randomized Controlled Trial

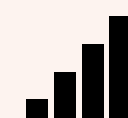
LGBTQ-
Affirmative CBT

vs

LGBTQ Affirmative
Community Therapy



Population: 450 Sexual minority
women in New York State



Ongoing!

“I used to think that being gay was a bad thing, and . . . that I should be ashamed of myself, that I should try and be straight, and that I shouldn’t hang out with, like, other gay people.” Now, it’s “the exact opposite.”

After the study, “I came out to my mom. I came out to a lot more friends. . . . Now, I’m just like—I’m just living my best life, as people say.”

“It helped me a lot to . . . how do I say it—be happy with myself, like accept myself. [In the sessions,] people . . . actually listened and cared about what I had to say.”

The sessions “made me feel like I wasn’t alone.”

Cultural Adaptations

Stressed? Lonely? Blue?



LGBTQ? 12-17 years old?

Participate in research study on LGBTQ teen mental health!

You might be eligible for 10 sessions of free online supportive group therapy and up to \$140

Yale LGBTQ
Mental Health
Initiative

Yale
SCHOOL
OF PUBLIC
HEALTH

To see if you're eligible, go to:
<https://tinyurl.com/YouthEQulPinfo>



PLEASE FEEL FREE TO CONTACT US
BY EMAIL: esteem.connect@yale.edu
BY PHONE: 475-441-3884

ESTEEM CONNECT

ARE YOU 18 YEARS OR OLDER?
DO YOU LIVE IN THE GREATER NEW HAVEN AREA?
DO YOU IDENTIFY AS A MAN OF COLOR AND
AS A MEMBER OF THE LGBTQ+ COMMUNITY?

IF SO, YOU MIGHT BE ELIGIBLE TO PARTICIPATE IN
A YALE UNIVERSITY PAID STUDY OF A SUPPORT
GROUP COUNSELING PROGRAM FOR QUEER MEN
OF COLOR WHERE YOU CAN EARN UP TO \$240.

Can LGBTQ Affirmative CBT Be Implemented in Community Settings?

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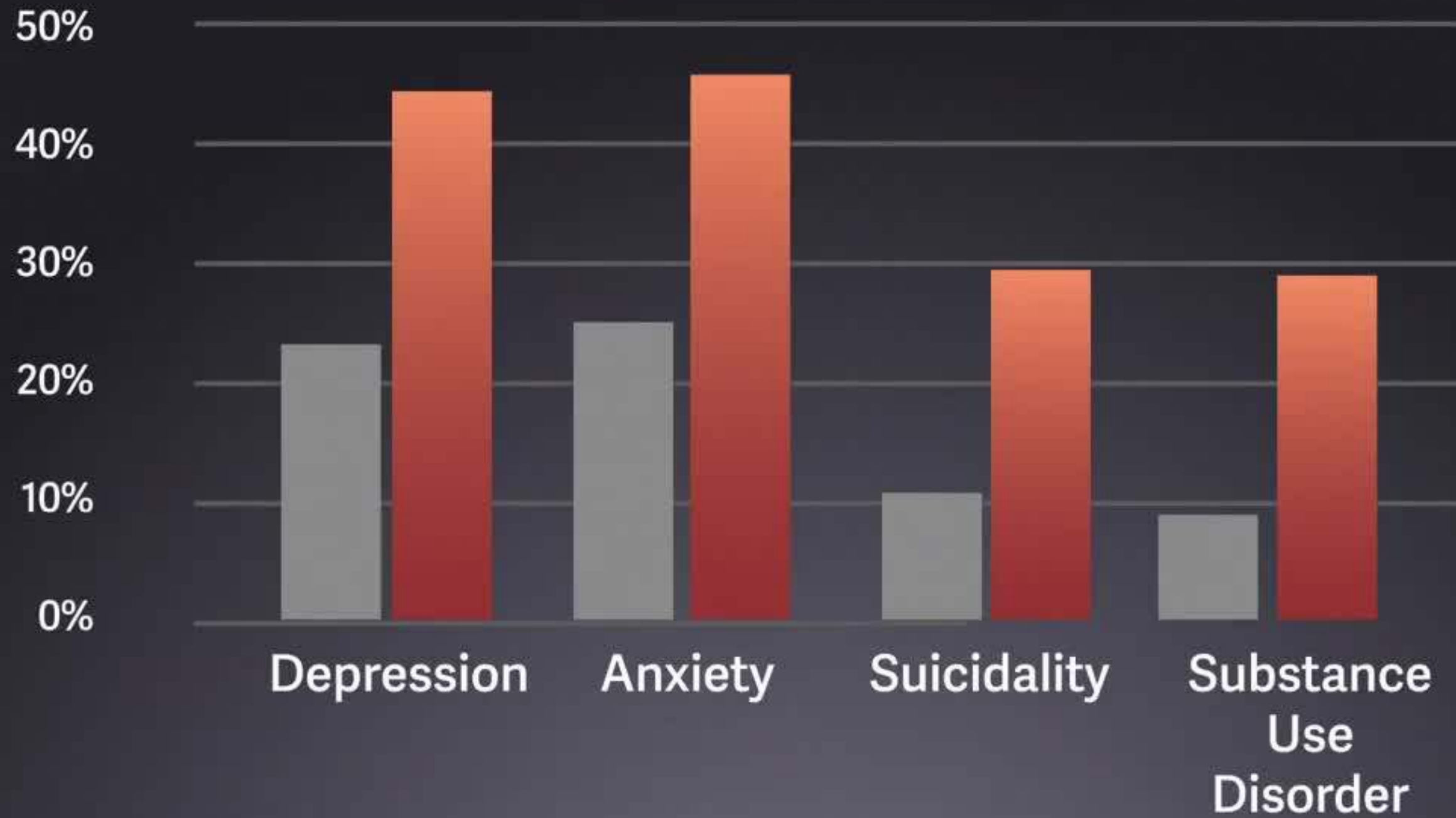
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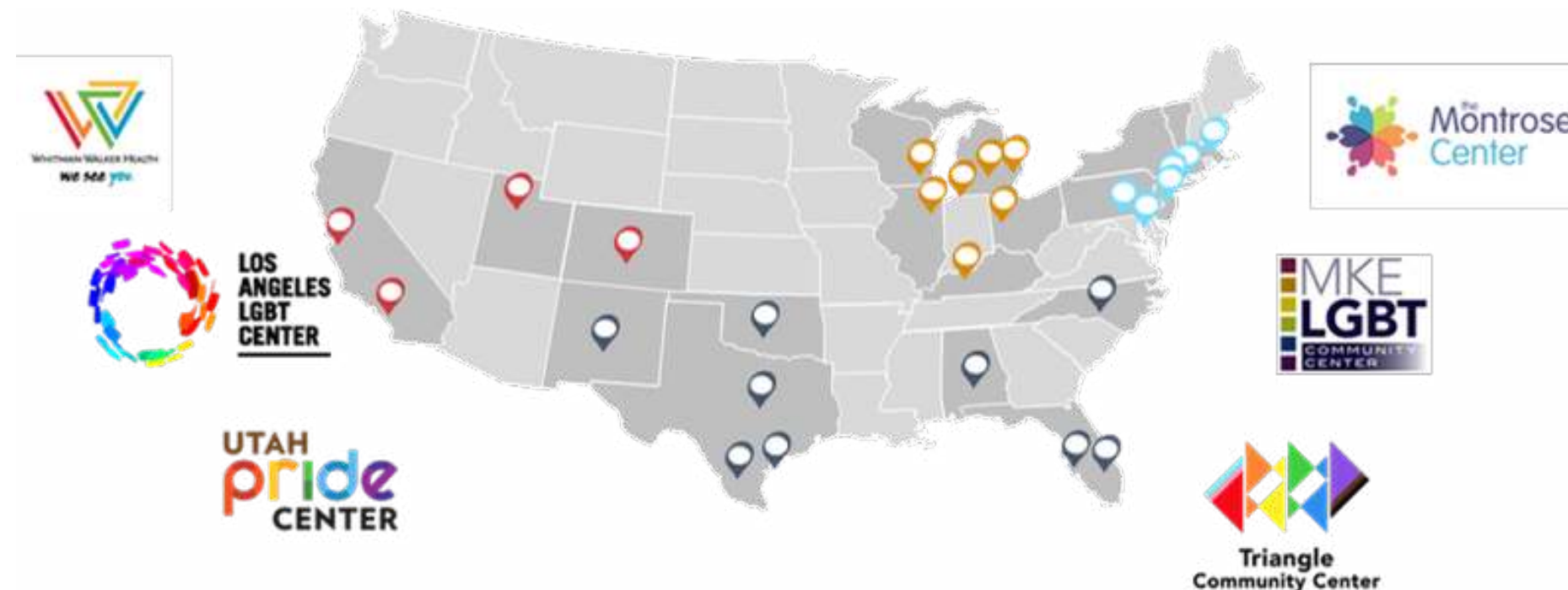
Expanding Reach into the Community

Minority Stress and LGBTQ-affirmative CBT

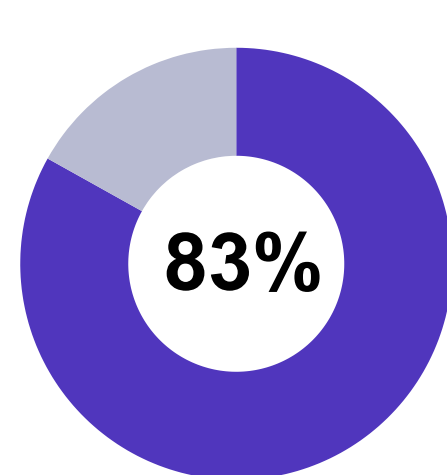


Community Implementation: Pilot

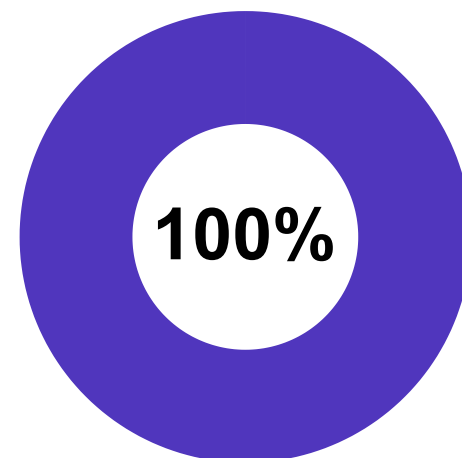
121 providers trained: 61 immediately trained and 60 trained after 4-mo wait



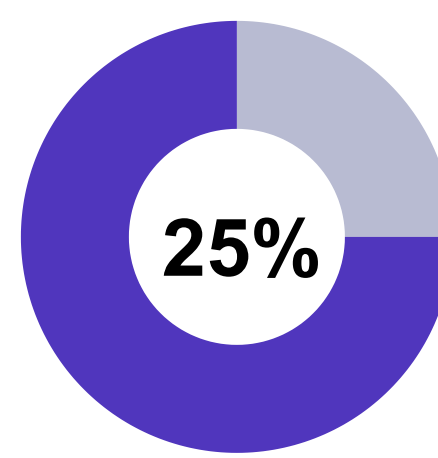
LGBTQ center CEOs and directors reported very high need and support for staff training in LGBTQ-affirmative CBT.



Thought staff would **benefit** from training

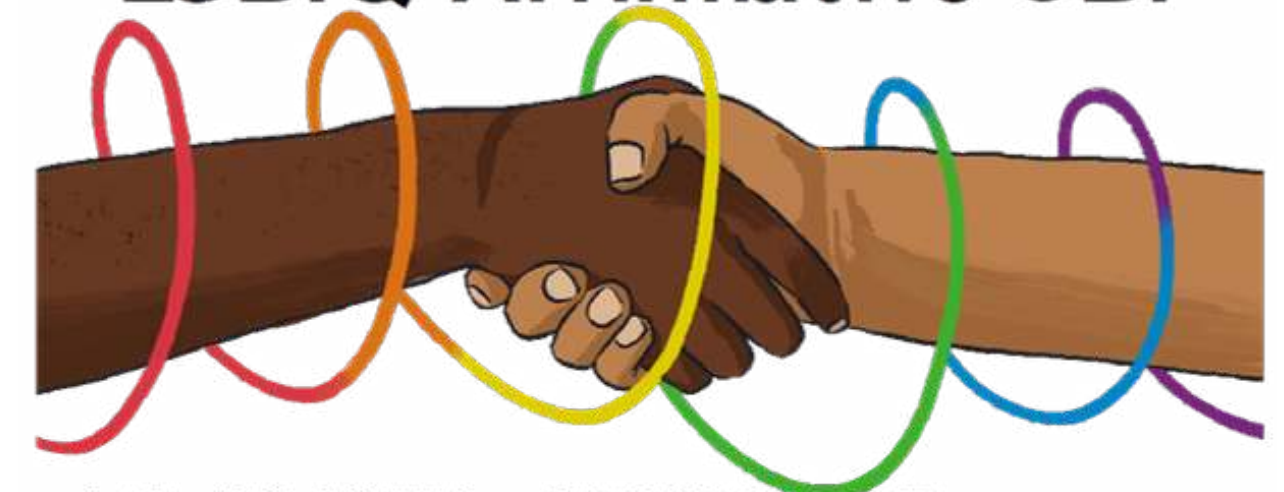


Supported staff **receiving training**



Thought they met **mental health needs**

Training Program in LGBTQ-Affirmative CBT



CenterLink and Yale School of Public Health are excited to offer the mental health staff of CenterLink members a comprehensive training in LGBTQ-affirmative cognitive behavioral therapy (CBT)! This training program consists of 11 weekly 1-hour seminars led by experts in state-of-the-art mental health care for the LGBTQ community. Participants will learn how to deliver a new type of evidence-based treatment that has been shown in federally-funded clinical trials to reduce depression, anxiety, substance use, and sex risk among diverse LGBTQ community members.

Trainers and Credentials:



Educational Objectives for Full Program:

- Apply CBT clinical skills specifically tailored to LGBTQ clients.
- Describe how minority stress theory can inform evidence-based mental health practice.
- Explain the principles and techniques of LGBTQ-affirmative CBT.
- Learning objectives for each session can be found here: <https://bit.ly/3kAtPF3>

Schedule and format:

All trainings will take place via live webinar on Zoom on Fridays from 12PM-1PM EST. Two date ranges for training (see back for more details):

1. Oct. 16, 2020 – January 22, 2021 (except Nov. 27, Dec. 25, Jan. 1)
2. March 5, 2021 – May 21, 2021 (except Apr. 2)

Appropriate level of skill & cost:

Intermediate, free

Participants for whom the activity is designed:

Mental health professionals and advanced graduate students at CenterLink community centers.

Continuing Education (CE) Credits Offered:

1 CE per session attended (up to 11 CE's total)

Register now for the course! —OR— Register for our October 2 informational webinar to learn more.

Division 44 is approved by the American Psychological Association to offer continuing education for psychologists. Division 44 maintains responsibility for this program and its content. Grievances regarding this program should be submitted directly to the Division 44 CE Coordinator, Jae Pickett, Ph.D., at jaepickett1@gmail.com

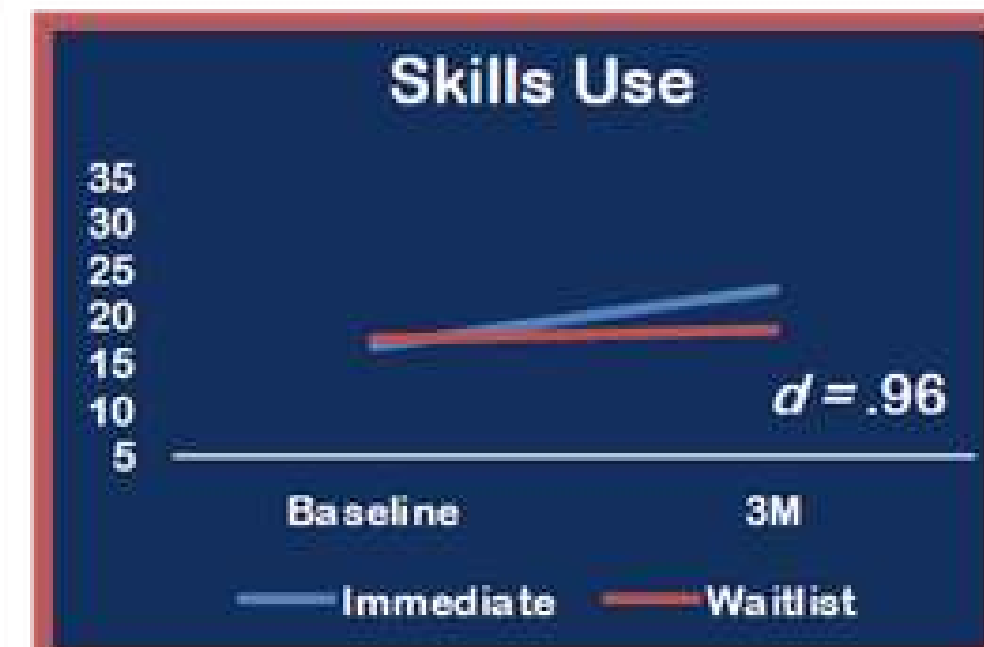
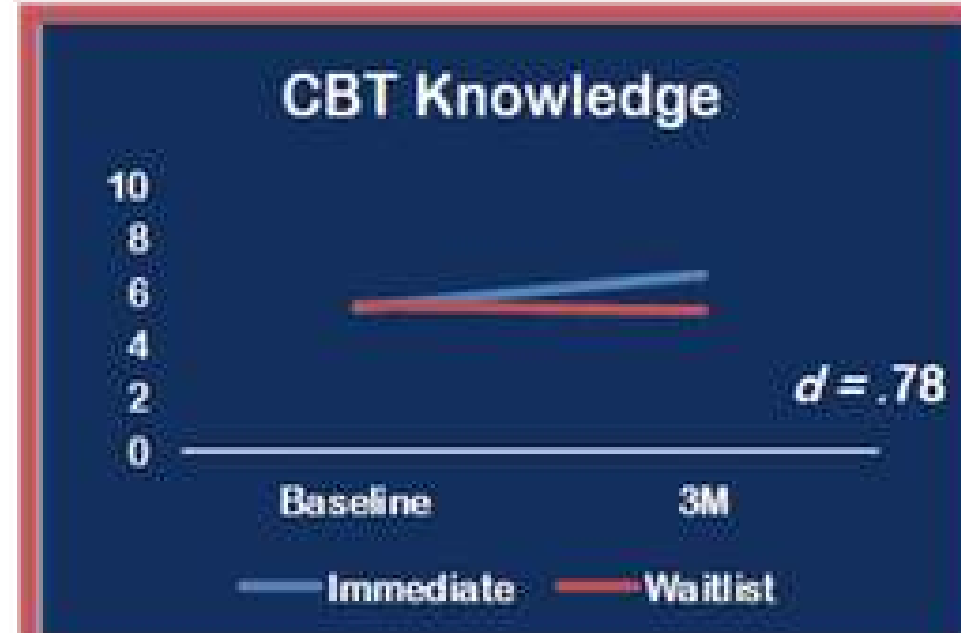
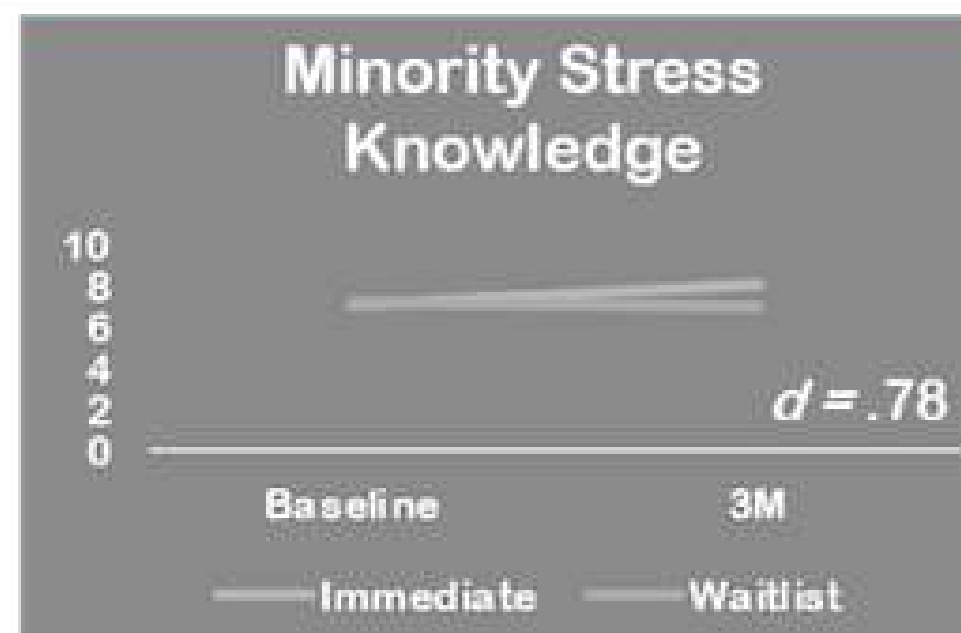
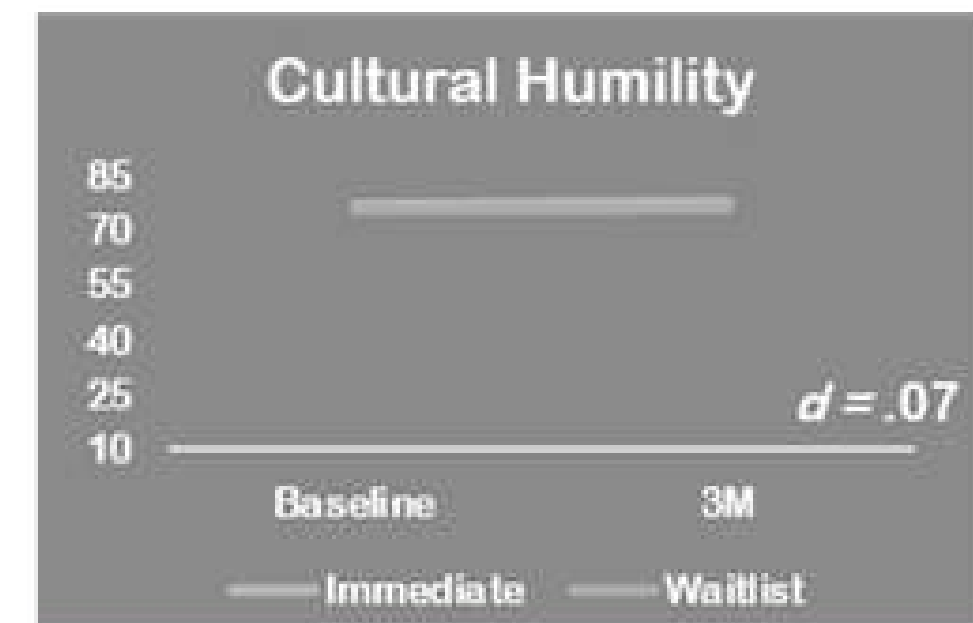
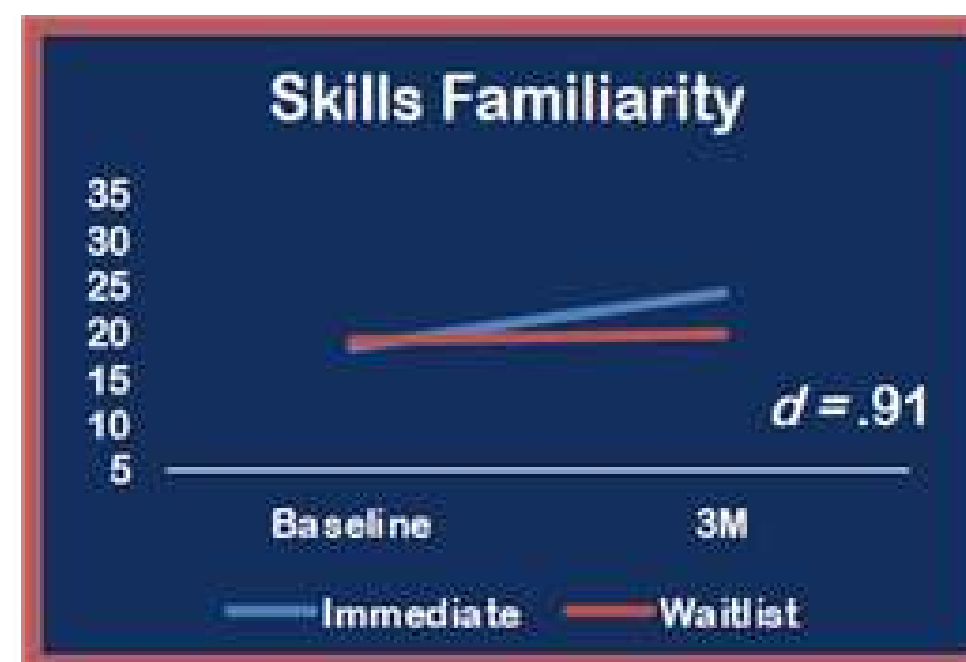
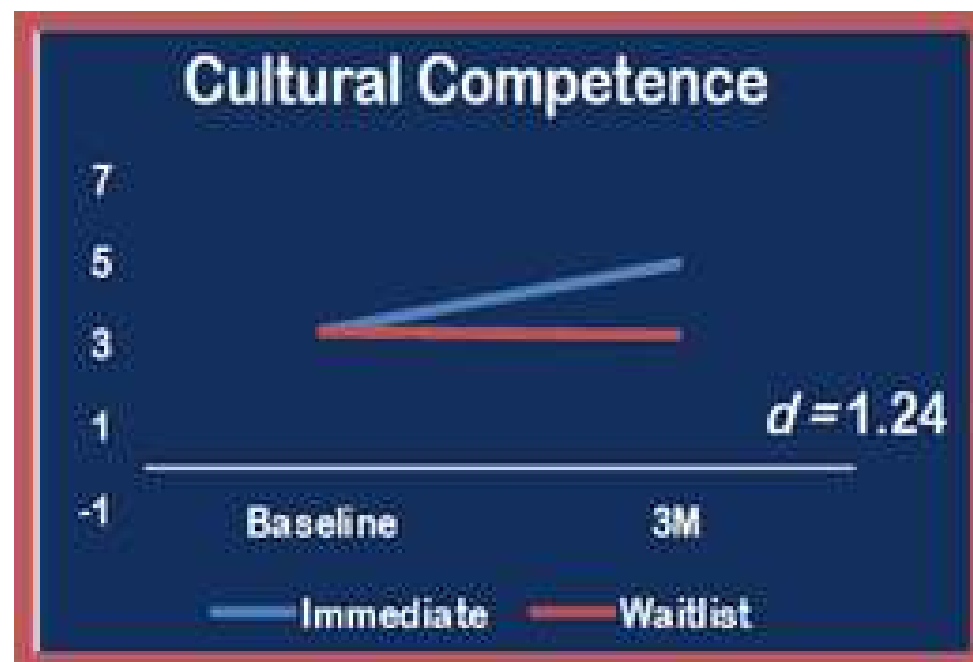


<https://bit.ly/338ll7a>

Community Implementation: Pilot

“The information I've learned throughout the training has given me a wealth of CBT exercises I feel confident utilizing with my LGBTQ clients.”

-Provider



Community Implementation: Full Trial

Testing strategies to train mental health providers in LGBTQ-affirmative CBT

Implementation Strategy

Implementation Strategy 1: Materials Only

Dashboard with provider and client materials

Implementation Strategy 2: Direct Training

Dashboard with provider and client materials

Implementation Strategy 3: Local Supervision

Dashboard with provider, client, and supervisor materials

+

For providers:

12 weekly training webinars

+

For providers and local supervisors:

12 weekly training webinars

+

For local supervisors:
3-day supervisor training followed by biweekly supervisor consultation

+

For providers:
At least monthly supervision by local supervisor

90

LGBTQ Centers across the US

540

Mental Health Providers serving LGBTQ clients

780

LGBTQ Clients

By the numbers

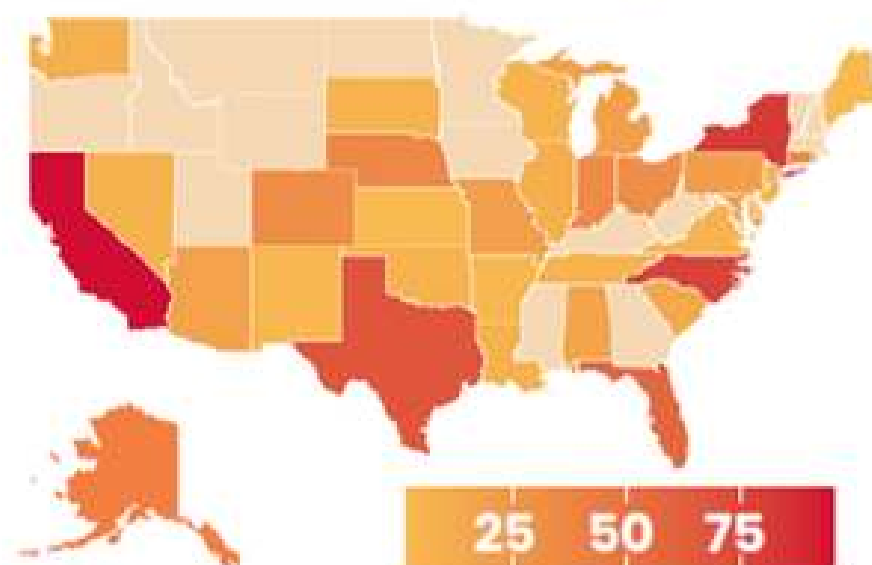
91

Centers enrolled

771

Providers gaining access to training

Providers Enrolled by State



If you haven't received your certificate or CE credits yet, please contact us!

663

Providers completed at least 1 training module

2300+

Hours of CE credit granted to date

479

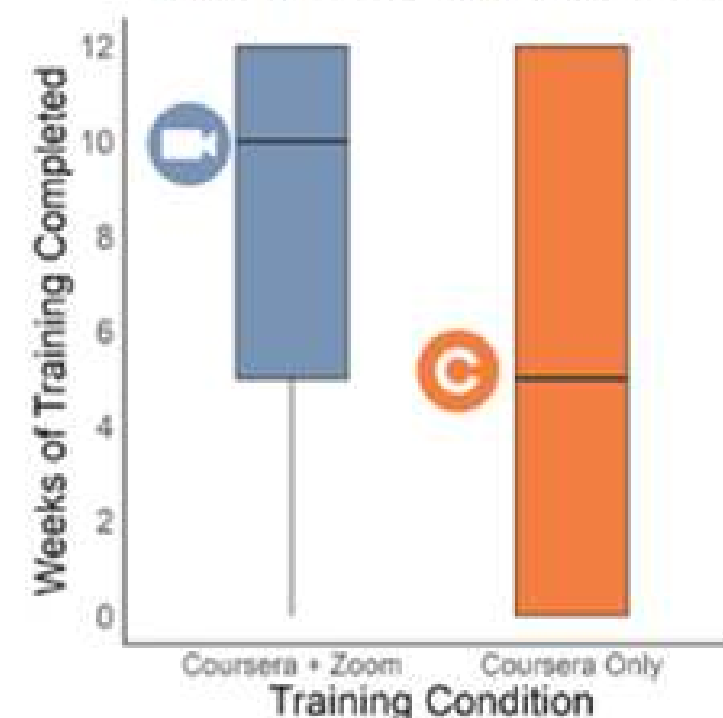
Providers earned a completion certificate

42%
Center Employees

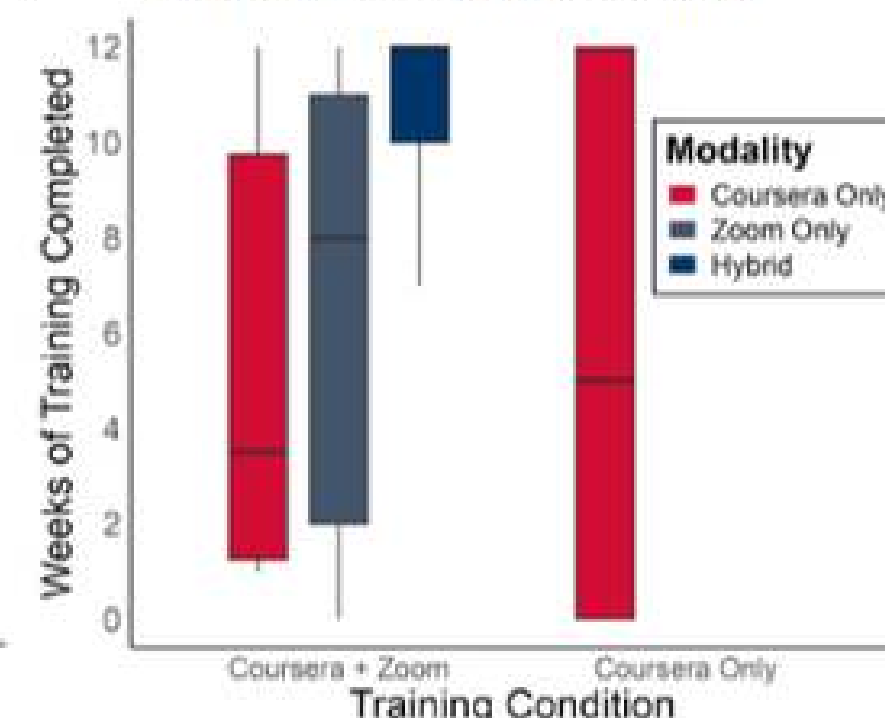
42%
Referral Providers

14%
Volunteers

Engagement by Condition



Engagement by Modality



- Engagement was higher ($t = -5.84$, $p < 0.001$) for those with access to zoom sessions ($M = 8.07$) than those without ($M = 5.75$).
- People who engaged with a mix of Coursera and Zoom completed the most weeks of training ($M = 10.69$)

240

only engaged with Coursera

323

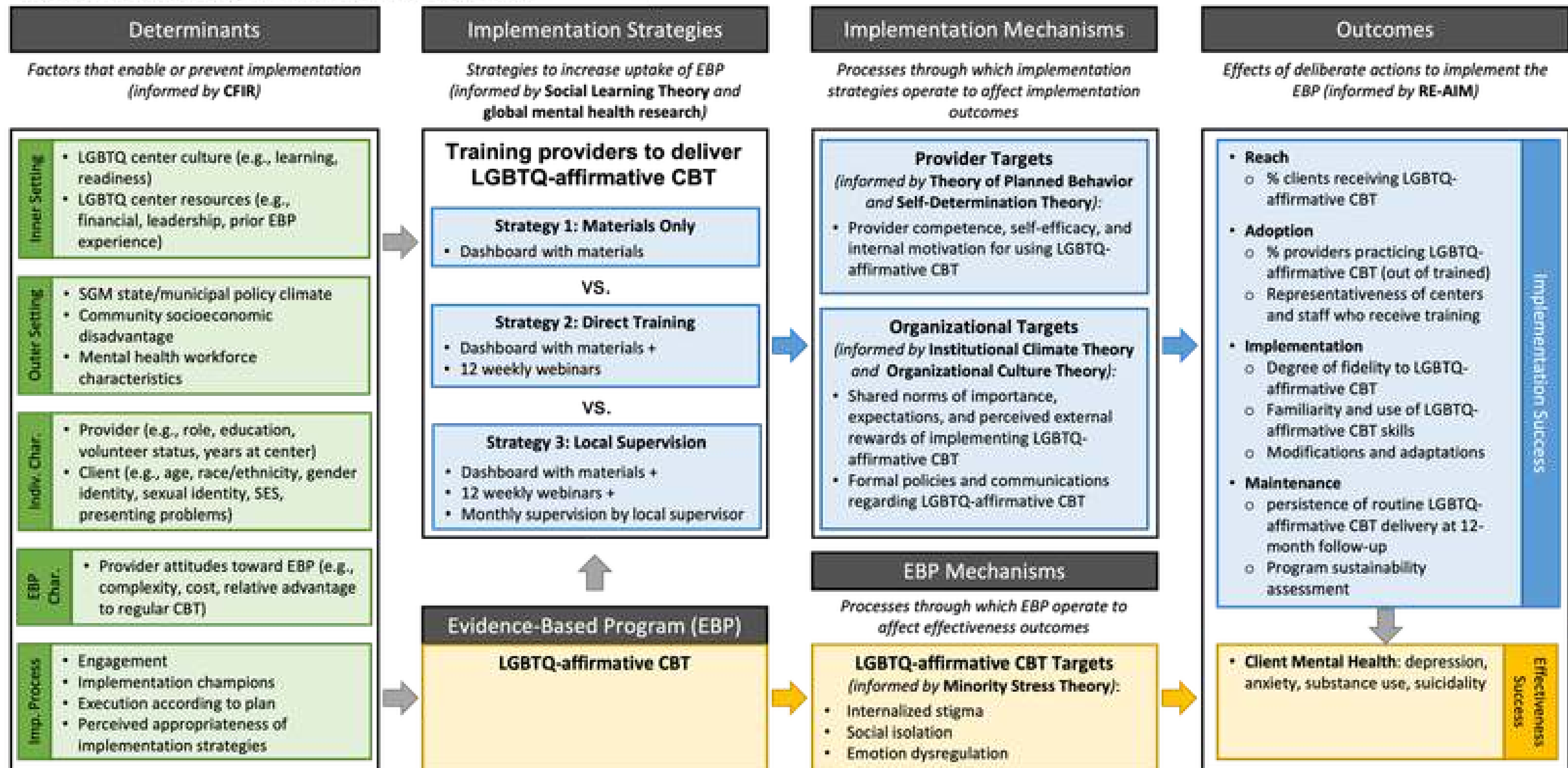
only engaged with Zoom

208

engaged with both (hybrid)

Community Implementation Trial

Figure 2. Study Logic Model and Key Constructs



Other Studies



Project SPACES

Project Goal:

- Strengthen connections between LGBTQ youth and their parents.

Activities:

- 3 brief online reflection exercises.
- 3 periodic surveys.
- 3 short conversations with research staff.

Objective: Foster better understanding and communication.

Currently recruiting parents of LGBTQ young adults (ages 15–29)



Project EMPATHS

Project Goal:

- Better understand therapy approaches for LGBTQ+ individuals and their parents

Activities:

- Psychotherapy sessions with SGM adults.
- Participant experience evaluations.
- Data analysis on therapy mechanisms

Objective: Improve psychotherapy approaches to support SGM communities better.

Currently recruiting LGBTQ young adults 20+

Community Extension

What are some simple, practical steps Guilford Youth & Family Services can take to create a more welcoming and supportive environment for LGBTQ+ youth, based on what we discussed today?

Questions?



Meredith McGee

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Yale LGBTQ Mental Health Initiative

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she/her/hers

Let's Stay in Touch!



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Twitter:

@YaleLGBTQhealth

Website:

